

VELA

Your clearest path to care.

PROJECT AND VIDEO IDEATION BRIEF

A self-contained guide to the product, technical system, user experience, NVIDIA story, demo truth, and strongest directions for a winning hackathon video.

DO TRACK

NVIDIA DGX SPARK

SEATTLE HEALTHCARE

Use this PDF as the source context for brainstorming the final 3 to 5 minute submission video in ChatGPT Work. It is accurate to the verified repository as of August 15, 2026.

Team VELA

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1. The project in one page

THE SIMPLE VERSION

VELA is a consent-controlled healthcare action system. A person explains what care they need, shares insurance evidence, and receives a source-backed comparison of realistic care paths. After the person explicitly approves each consequential action, VELA can carry a sandbox workflow through enrollment, coverage transition, provider verification, appointment booking, and auditable receipts.

THE APPROVED POSITIONING

VELA turns a spoken care request and insurance documents into a verified, cost-aware care path, then, with the user's explicit approval, takes action to secure the appointment. Under the interface is a multistep agentic workflow combining local NVIDIA reasoning, document retrieval, deterministic matching, annual-cost calculations, tool use, recovery logic, and an auditable record of every consequential action.

THE HUMAN PROBLEM

People navigating care must decipher insurance documents, confirm eligibility and networks, estimate fragmented costs, call providers, coordinate coverage changes, and schedule appointments. The difficult part is not finding information. It is turning incomplete, contradictory information into a safe sequence of decisions and actions.

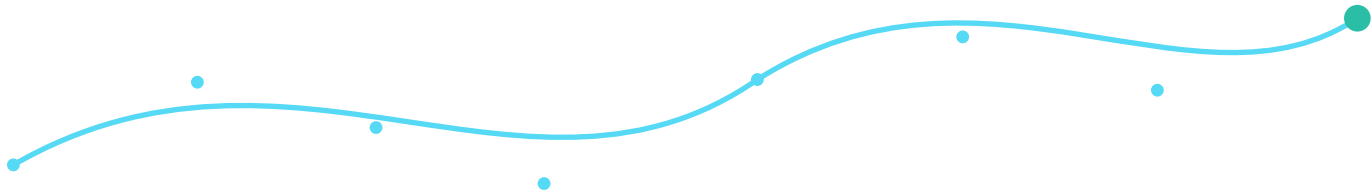
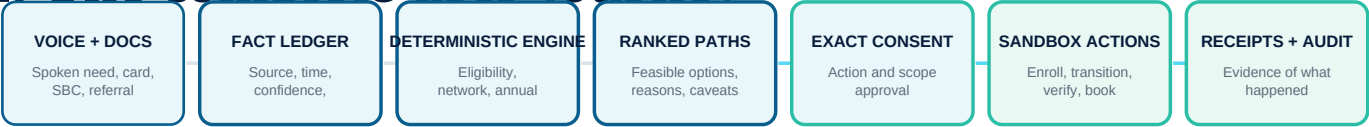
THE FEELING VELA SHOULD CREATE

- Clarity: the user can finally see the path through complexity.
- Control: nothing consequential happens without exact permission.
- Confidence: recommendations show reasons, costs, sources, caveats, and rejected alternatives.
- Relief: the system carries coordination work forward instead of handing back a list of links.
- Trust: uncertainty and sandbox boundaries are visible rather than hidden.

WHY THE NAME WORKS

VELA evokes navigation, direction, and forward movement. The visual system turns healthcare complexity into branching paths that begin dormant, illuminate as agents validate evidence, and resolve into one bright recommended route.

2. The complete user journey



THE GOLDEN DEMO STORY

A fictional Washington resident recently lost employer coverage and needs a nonemergency knee MRI. They want to preserve a required medication and preferred physician while minimizing expected annual cost. VELA compares continuation coverage with two eligible alternatives, selects the lowest-cost feasible care path, obtains exact approvals, and completes a sandbox appointment workflow.

Stage	What the user sees	What the system proves
1. Onboard	Voice or text conversation; a progressive question at a time	A spoken need becomes a structured journey without a questionnaire dump
2. Add evidence	Camera scan or PDF upload for card, SBC, and referral	Candidate facts retain document source and verification status
3. Clarify	A question when “knee MRI” is underspecified	VELA refuses to guess a billing code
4. Compare	Branching care paths awaken as agents work	Eligibility, network, and annual cost are deterministic
5. Recommend	One path becomes brighter than every alternative	The best feasible option includes reasons, sources, and caveats
6. Consent	Separate approval screens for each action and scope	Analysis is not permission to act
7. Execute	Sandbox enrollment, transition, verification, and booking	Adapters are stage-bound and idempotent
8. Prove	Appointment summary, receipts, and audit timeline	Completion is evidenced, not merely claimed

3. What the product looks and feels like

VISUAL IDENTITY

A calm, luminous, premium interface: warm white space, deep navy typography, restrained blue and teal, fine paths, soft particles, and a single bright cyan route. It should feel serious enough for NVIDIA and reassuring enough for someone making an important care decision.

THE SIGNATURE INTERACTION

- The neural care map does not appear before the conversation begins.
- After the user speaks or sends a message, faint branching paths emerge behind transparent controls.
- Every agent handoff reveals nodes, branches, and pulses of activity.
- Paths remain deliberately muted while evidence is incomplete or options are still being tested.
- Once deterministic evaluation identifies the best feasible path, one continuous line illuminates brighter than all previous activity and remains lit through consent and booking.

WEBSITE EXPERIENCE

- Persistent desktop navigation: Home, Paths, Appointments, Documents, Preferences.
- Voice and chat are two modes of the same journey, not separate products.
- Transparent chat composer, document controls placed to the side, and no resizing or page jump when a message is sent.
- The animated path system occupies the conversation canvas and responds to each turn.

IPHONE EXPERIENCE

- The same visual identity adapted to native touch and safe areas.
- Camera flow for insurance cards and referrals, plus PDF/image upload.
- Fixed mobile navigation with stacked decision, consent, and receipt cards.
- Call and chat show the same path-awakening behavior and final bright route.

FUNCTIONAL WORKSPACE TABS

Tab	Purpose
Paths	Ranked options, feasibility, reasons, sources, rejected alternatives, and annual-cost components
Appointments	Proposed slots, booking approval, confirmed sandbox appointment, and rescheduling flow
Documents	Camera capture, file upload, document classification, extracted facts, provenance, and review state
Preferences	Physicians, medications, travel radius, scheduling, accessibility, language, and comfortable cost limits

4. How VELA works

ONE ENGINE OWNS THE TRUTH

The Care Journey Engine owns workflow state and authoritative decisions. Agents are bounded capabilities around it. This separation is central to the project: the model can understand and explain language, but deterministic application code decides what is feasible and whether an action is permitted.

Layer	Responsibility	Cannot do
Voice, chat, camera, PDFs	Normalize user intent and evidence	Decide eligibility or authorize an action
Onboarding and extraction agents	Propose facts, classify documents, identify missing information	Silently verify facts or invent absent benefits
Fact and memory ledger	Append facts with source, time, confidence, and status	Erase history or promote uncertain facts
Plan and provider catalogs	Retrieve controlled Seattle/Washington records	Recommend a path by themselves
Deterministic engine	Eligibility, network rules, annual cost, ranking, consent, state transitions	Delegate authority to model prose
Nemotron explanation agent	Explain the engine evidence in plain language	Change costs, ranking, consent, or state
Sandbox adapters	Simulate actions for an approved exact scope	Run without consent, correct stage, and idempotency key
Audit ledger	Record facts, approvals, failures, transitions, and receipts	Rewrite history

ANNUAL-COST LOGIC

Annual premiums + expected out-of-pocket care + medication costs + remaining deductible exposure. The cheapest MRI price is not necessarily the cheapest complete care path.

MODEL OUTPUT IS UNTRUSTED INPUT

Every Nemotron response is validated against a closed schema. Unknown actions are rejected. One bounded corrective retry is allowed; repeated failure becomes a visible blocked or operator-review state.

5. The NVIDIA and DGX Spark story

THE LOCAL INFERENCE PATH

VELA application -> Care Journey Engine -> authenticated Hermes gateway -> NemoClaw/OpenShell policy boundary -> NVIDIA Nemotron 3.5 Lightning -> NVIDIA GB10 on an Acer Veriton GN100.

The application does not call vLLM directly. Tailscale and SSH protect host access. Hermes authenticates the application. NemoClaw controls model and network execution. VELA separately enforces healthcare evidence, consent, and workflow rules.

WHY LOCAL NVIDIA COMPUTE MATTERS

- Sensitive insurance and care context can remain inside a local trust boundary.
- The model, retrieval context, document processing, and long-running workflow state can operate together on one system.
- Predictable conversational latency makes voice onboarding and explanations feel responsive.
- Deterministic comparison continues if Nemotron is unavailable; language-dependent steps fail closed or request review.
- The project demonstrates NVIDIA as operational infrastructure for an agent that acts, not a decorative model call.

WHAT NEMOTRON DOES

- Extracts candidate facts from speech and documents.
- Classifies closed-set intents and document types.
- Normalizes procedure, plan, provider, and facility names.
- Detects ambiguity and proposes clarification.
- Explains authoritative comparisons and summarizes audit history.

WHAT NEMOTRON NEVER DOES

- Calculate authoritative costs or decide eligibility/network status.
- Rank plans or providers.
- Mark evidence verified, grant consent, advance workflow state, or execute actions.

VIDEO PROOF TO CAPTURE

- A brief shot showing local Nemotron inference through the Hermes/NemoClaw route.
- A latency measurement from request to validated response.
- If available, GB10 utilization or memory evidence during the model step.
- One transition from model-proposed language to deterministic evidence and grounded explanation.
- Never expose bearer tokens, SSH details, private hostnames, or raw sensitive payloads.

6. Consent, safety, and recovery

THE CORE TRUST PROMISE

VELA may research and calculate after document-processing permission, but it may not enroll, change coverage, disclose information to a provider, or book an appointment without a separate approval for the exact action and scope.

Gate	What must be approved	Why separate
Document processing	The specific documents and analysis purpose	Uploading evidence is not permission to share or act
Enrollment	Selected sandbox plan and enrollment submission	Choosing an option is not enrollment authority
Coverage transition	Replacement effective date and confirmed first premium	Old coverage must not end before replacement is secure
Provider disclosure	Minimum information and named provider purpose	Limits unnecessary disclosure
Appointment booking	Provider, location, time, and appointment request	A recommendation is not permission to reserve

RECOVERY IS PART OF THE PRODUCT

Failure	Safe behavior	Recovery
Ambiguous MRI	No guessed code	Ask with or without contrast
Missing benefit	Estimate remains incomplete	Request SBC or bound the result
Out of network/ineligible	Path cannot win	Show the hard rejection reason
Invalid model schema	Reject output	One retry, then operator review
Hermes unavailable	Preserve deterministic result	Label explanation unavailable or retry locally
Missing consent	Do not call adapter	Explain scope and wait
Duplicate request	Do not repeat action	Return existing receipt
Adapter failure	Never claim completion	Record failure and retry safely

7. What is real, sandboxed, and prohibited

WORKING IN THE PROTOTYPE

- Seeded insurance-card and referral extraction.
- Personal Care Twin / fact-and-memory state.
- Three-plan deterministic cost comparison.
- Special Enrollment eligibility gate.
- Exact consent ledger.
- Provider verification through a controlled endpoint or call.
- Complete responsive VELA interface with voice, chat, camera/PDF intake, paths, appointments, documents, and preferences.
- 55 passing deterministic and vertical-slice tests.

SANDBOXED

- Plan enrollment submission.
- Existing-coverage transition.
- Appointment booking.

NEVER CLAIM

- That VELA changed real insurance or canceled real coverage.
- That a sandbox booking reserved a real clinical appointment.
- That an estimate is a guaranteed patient price.
- That VELA diagnosed a condition, recommended treatment, or acted as a licensed broker.
- That model-generated language is the source of eligibility, cost, ranking, or consent decisions.

DATA POLICY

The judged journey uses a fictional Washington persona and controlled synthetic card, SBC, referral, plan, provider, and action data. The repository contains no real patient or insurance documents. A card can identify visible plan fields but cannot establish a full benefit design; cost-sharing evidence requires an SBC or equivalent source.

FINAL DEMO STATE

New sandbox coverage selected, MRI sandbox booking completed, expected annual savings quantified, and every material action visibly approved and sourced.

8. Why VELA can win the Do Track

THE TRACK THESIS

VELA does not stop at answering a healthcare question. It orchestrates a long-running workflow across voice, documents, plans, providers, costs, consent, and booking tools to accomplish a task that a patient would otherwise coordinate manually.

Judging signal	VELA evidence	Best video proof
Workflow completeness	Intake through sandbox appointment and audit receipt	Show the whole golden path, compressed but continuous
Branching and recovery	Ambiguity, rejection, consent blocking, transition prerequisites, retries	Use one short "I need a knee MRI" clarification moment
Technical depth	Ledger, catalogs, deterministic engine, bounded agents, state machine, adapters	Show one architecture frame and one live audit trace
NVIDIA ecosystem	Nemotron through authenticated Hermes/NemoClaw on GB10	Show local inference and explain the trust/latency reason
Human value	Complex insurance work becomes one clear path to care	Keep the fictional patient goal present throughout
Usability	Voice/chat, neural paths, mobile/web, camera/PDF, exact consent	Let the path animation communicate invisible agent work
Performance	Local language inference plus fast deterministic calculation	Overlay honest measured latency or utilization

THE DIFFERENTIATOR IN ONE SENTENCE

Most healthcare assistants return information; VELA turns verified evidence into a safe, consent-controlled sequence of actions and receipts.

THE MEMORABLE VISUAL METAPHOR

Many muted possibilities become one illuminated route. The visual is not decoration: it mirrors the engine evaluating constraints, rejecting infeasible options, and converging on the clearest feasible path.

9. Video ingredients and possible hooks

HOOK DIRECTIONS TO IDEATE

Direction	Opening idea	Emotional promise
The maze	"Getting an MRI should not require becoming an insurance expert."	VELA turns a maze into one clear route
The permission problem	"AI can act. The harder question is when it should be allowed to."	VELA acts only after exact consent
The invisible work	Show the dormant network waking as the user speaks	The user can finally see coordination happening
The wrong cheapest option	Start with a cheap procedure price, then expose total annual cost	VELA optimizes the whole care path
The local trust boundary	Start on the DGX Spark and follow one request through the stack	Sensitive context stays local and policy controlled

MOMENTS WORTH SHOWING

- The first spoken sentence and the first faint paths appearing.
- Camera scan or PDF upload beside the transparent conversation.
- VELA refusing to guess whether the MRI needs contrast.
- Several paths activating while agents work.
- One route becoming dramatically brighter at recommendation.
- The visible annual-cost breakdown and rejection reason for another option.
- An exact consent card stating precisely what VELA will do.
- A sandbox receipt and final appointment view.
- A compact audit trace connecting evidence, approval, action, and receipt.

WHAT TO KEEP OFF SCREEN

- Long terminal setup, dependency installation, and ordinary navigation.
- Tokens, private infrastructure, real health data, or claims beyond the demo truth.
- Dense architecture diagrams for more than a few seconds.
- Every edge case; demonstrate one and mention the rest.

10. A strong 3 to 5 minute structure

Time	Story beat	What appears
0:00-0:20	Human hook	One sentence problem; dormant VELA interface
0:20-0:45	Product promise	Voice request, path network begins to awaken
0:45-1:20	Evidence intake	Synthetic card camera/PDF; source-backed facts
1:20-1:45	Responsible intelligence	MRI ambiguity clarification; no guessed code
1:45-2:30	Decision engine	Three paths evaluated; cost components and hard constraints
2:30-2:50	Signature reveal	One route illuminates brighter; reasons and caveats
2:50-3:35	Controlled action	Exact consent, sandbox adapter, receipt, appointment
3:35-4:05	Technical proof	Care Journey Engine, Nemotron boundary, DGX Spark/Hermes
4:05-4:30	Recovery and truth	One blocked state; sandbox/estimate caveat
4:30-5:00	Outcome and close	Savings, booked sandbox appointment, audit history, tagline

PRESENTATION PRINCIPLES

- Make the user story the spine. Technical details should explain why the experience is trustworthy.
- Show cause and effect: user input -> agent activity -> validated evidence -> deterministic decision -> consent -> receipt.
- Use measured numbers only. If latency or savings is not verified, label it illustrative or omit it.
- Say “sandbox” at the moment of action, not only in fine print.
- End on the completed care path and human relief, not on architecture.

11. Copy this into ChatGPT Work

PHONE BRAINSTORMING PROMPT

I am preparing a 3 to 5 minute NVIDIA Spark Hack Series Do Track submission video for VELA. Use the attached VELA project brief as the authoritative project context. Help me brainstorm three genuinely different video concepts that maximize our chance of winning.

For each concept, provide: (1) the emotional hook, (2) a second-by-second story structure, (3) exact scenes and screen recordings, (4) narration draft, (5) what NVIDIA/DGX Spark proof appears, (6) where the neural care-path animation becomes the signature moment, (7) how we demonstrate one recovery branch, (8) how we state sandbox and estimate limitations honestly, and (9) why this concept is strongest for the Do Track.

Do not invent product capabilities, metrics, external integrations, real enrollment, real booking, or guaranteed prices. Ask me for any latency, savings, or utilization number before placing it in narration. Keep the user story emotionally clear and make the technical architecture support the story rather than overwhelm it. After showing the three concepts, recommend one and explain the tradeoffs.

FOLLOW-UP QUESTIONS TO USE

- Which first 15 seconds would make a judge stop multitasking?
- How can we visually connect local Nemotron inference to the care path without showing terminals for too long?
- Which single recovery case best proves technical maturity?
- What should the user say aloud so the narration feels human rather than scripted?
- Where should we cut between website, iPhone, and architecture without confusing the story?
- What exact phrase should end the video and make VELA memorable?

12. Current readiness and next actions

Requirement	Status	Next action
Team name: VELA	Ready	Enter exactly in form
Project description	Ready	Use approved positioning from this brief/README
Do Track rationale	Ready	Use the workflow-completeness thesis
Repository	Readiness PR open	Abhinav merges PR #3 into main
Deployed application	Ready	Verify signed out: vela-care-path.fsaguilar16.chatgpt.site
Team roster and contacts	Ready	Copy into submission form
Engineering verification	Ready	55 tests, typecheck, builds, clean clone, secret scan passed
Live NVIDIA proof	Pending	Capture Hermes/Nemotron route, latency, optional GB10 utilization
Rehearsals	Pending	Run the golden journey three times after merge
Demo video	Pending	Record 3 to 5 minutes, upload unlisted, verify signed out
Final review	Pending	Second teammate checks every link and claim
Submission	Pending	Submit before August 16, 11:00 AM Pacific; save receipt

TEAM

Member	Role	Contact
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LINKS

- Application: <https://vela-care-path.fsaguilar16.chatgpt.site/>
- Repository: <https://github.com/abhinavm27/abyss>
- Readiness PR: <https://github.com/abhinavm27/abyss/pull/3>

13. One-page filming cheat sheet

THE STORY

A fictional person needs a knee MRI after losing employer coverage. VELA listens, reads synthetic insurance evidence, refuses ambiguity, compares full annual cost across feasible paths, illuminates one best route, asks exact permission, and completes a sandbox booking with receipts.

THE THREE IDEAS JUDGES SHOULD REMEMBER

- VELA acts: it carries the journey from need to controlled outcome.
- VELA is trustworthy: models understand language; deterministic code owns decisions; users own permission.
- VELA runs local NVIDIA intelligence inside a layered trust boundary designed for sensitive, long-running work.

THE SIGNATURE SHOT

Muted branching paths awaken during the conversation. At the recommendation, one cyan route becomes brighter than anything before it and stays illuminated through the final receipt.

THE SAFETY LINE

“This prototype uses controlled synthetic data and sandbox adapters. Every consequential action requires separate, exact consent.”

THE CLOSE

“VELA. Your clearest path to care.”

FINAL CHECK BEFORE RECORD

- Reset the synthetic scenario.
- Hide tokens, host details, notifications, and unrelated tabs.
- Confirm voice/chat, camera/PDF, path animation, consent, receipts, and audit view.
- Record NVIDIA proof and fallback footage separately.
- Use only verified metrics and claims.
- Verify the exported video signed out before submission.